

tion disclosed normal foveal concavity overlying healthy retinal pigment epithelium (Figure 2, bottom).

This is the first report to our knowledge to describe excision of a neovascular membrane during foveal translocation surgery without large retinotomy. A small retinotomy site near the fovea for excision did not interfere with creation of an intentional retinal detachment. After translocation, the fovea recovered both anatomically and functionally, with a normal concave structure situated over intact retinal pigment epithelium. This anatomic configuration was confirmed with the aid of optical coherence tomography.

We have shown that foveal translocation with scleral shortening and simultaneous excision of a neovascular membrane can lead to the recovery of the foveal structure and function. Because scleral shortening allows relatively small shifts in foveal position, this procedure may be most useful in patients with age-related macular degeneration and small subfoveal neovascular membrane.

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Three-Dimensional Ultrasonography of Choroidal Melanoma: Extrascleral Extension

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PURPOSE: To describe the results of three-dimensional ultrasonography used to evaluate extrascleral extension of a choroidal melanoma.

METHODS: Case report. The three-dimensional ultrasound system uses a 10-MHz B-mode transducer combined with a motorized rotating holder. The system acquires 180 sequential images that are stored and processed to create a three-dimensional block of the region of interest.

RESULTS: Unique coronal and oblique perspectives were obtained from interactive manipulation of the three-dimensional reconstruction. Examination of the three-dimensional image allowed us to detect the transscleral uveal–orbital connection. Extrascleral melanomatous extension was confirmed on histopathologic examination.

CONCLUSION: Three-dimensional ultrasonography is a promising imaging technique for evaluating melanomatous extrascleral extension. (*Am J Ophthalmol* 1998;126:842–844. © 1998 by Elsevier Science Inc. All rights reserved.)

EXTRASCLERAL EXTENSION OF CHOROIDAL MELANOMAS affects both management and prognosis.^{1,2} Although this condition is usually treated by enucleation or exenteration, it is helpful to know the extent of orbital disease before surgery. Conventional ultrasound and computed tomography have been used to detect and evaluate extraocular tumor extension.^{3,4} A new method, computer-assisted three-dimensional ultrasonography, can also be used for these difficult cases.⁵ In this case, a patient suspected of having melanomatous extrascleral extension was imaged with conventional B-scan ultrasonography, computed tomography scan, and three-dimensional ultrasonography.

Three-dimensional ultrasonography was performed using the three-dimensional i-scan (Ophthalmic Technologies Inc, Downsview, Ontario, Canada), which is commercially available. This system uses a 10-MHz B-mode transducer combined

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